INDUSTRY STARTER PACKS FOR ENTERPRISE ARCHITECTS

Healthcare

The learner guide — what is in it, and the order to pick it up

A 259-page guide to an industry where downtime is clinical, the record is three instruments at once, and almost nothing that matters is specified nationally. Read this first: it says which chapter answers which question, and what the book does when it cannot establish something.

South African lens. Sourced throughout. October 2026.

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Thotnr Consulting

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GUIDE 1

What is in this delivery

One book. Three of its twenty-five items are reference, not reading.

This book exists so that an architect who has never worked in healthcare can walk into a hospital group or a provincial health department and be useful. Not eventually — within a quarter.

It is the first part of a larger set. **What ships here is the learner guide and this index.** The workbook, the six spreadsheets and the three decks are built, in this series, after the guide; they are not in this delivery. What that means practically is in the last section.

Read this first

> IF YOU READ NOTHING ELSE
>
> `01 Learner guide` — read it once, front to back, before you start anything. It is the industry. 259 pages, 25 items, 114 figures, 865 sourced citations.
>
> Then keep it beside you. Four parts of it are reference rather than reading: the glossary, the cross-maps, the standards appendix, and chapter 2.

What is in the guide

Part	What it answers	Chapters	Page
I — The industry	What this industry is, and how to read its numbers	1 to 4	3
II — How a provider runs	Who does what, how the money moves, and what the clock looks like	5 to 9	42
III — Data and systems	The model, identity, coding, the record, the estate and the seams	10 to 16	99
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Chapter by chapter

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Which chapter answers which question

If you need to	Go to
Understand the industry in an hour	Chapter 1
Quote a figure in a steering committee	Chapter 2, first
Work out who you actually need in the room	Chapter 5
Explain why a claim was rejected	Chapter 6
Answer "what does NHI mean for us"	Chapter 7
Plan a release against a clinical calendar	Chapter 8
Model a process that crosses four organisations	Chapter 9
Draw the data model	Chapters 10 and 4
Argue about patient matching	Chapter 11
Decide a coding or terminology question	Chapter 12
Answer a records, retention or access question	Chapter 13
Describe the estate to someone who has not seen it	Chapters 14 and 15
Write an integration specification	Chapter 16
Prepare for a vendor meeting	Chapter 17
Answer a POPIA or regulator question	Chapter 18
Set a recovery objective, or argue for one	Chapter 19
Model any of it in ArchiMate 4	Chapter 20
See what a finished repository looks like, defects and all	Chapter 21
Work out what to do on Monday	Chapter 22

Six things in here that will surprise an architect from another industry

1. **Downtime is clinical.** Not a service-level matter. The planning assumption for a serious outage in this sector is weeks, not hours, and the book names the incidents that establish it.
2. **The strongest business case is medico-legal, not clinical.** In an audit of 27 facilities, three of nine paid claims were conceded because nobody could find the medical record. Provinces pay around R1,5bn a year.
3. **Almost nothing that matters is specified nationally.** No national procedure classification, no national tariff since 2010, no national medicines dictionary, no present-on-admission flag, no national identity-merge policy. The architect's job is making those decisions locally and writing them down.

4. **The sector's own official numbers do not reconcile**, and the disagreements are large, recent and published by the same institutions. Chapter 2 teaches this as a discipline.
5. **A measured South African failure of record linkage exists** and it is worse than anyone assumes: in 13,499 infant tests, 4% of infants matched exactly on all four demographic details.
6. **NHI is a contingency that validates the work, not the reason for it.** No section is in force, and everything an NHI architecture would require is already mandated by instruments in force today.

How this book handles what it does not know

The research behind it runs to 219,000 words across 869 sources, and it carries **217 items that could not be verified**. Where something could not be established, the book says so in the text and names what would settle it, rather than estimating. Where two official sources disagree, it prints both and says they disagree.

> TWO THINGS TO RE-CHECK BEFORE YOU RELY ON THEM
>
> **The Constitutional Court's NHI judgment** — heard 5 to 7 May 2026, still reserved at the end of October 2026.
>
> **The SNOMED CT National Release Centre for South Africa** — the country joined SNOMED International on or about 14 May 2026, and no release centre was operating at the time of writing. If one now is, chapter 12 changes.

Two invented organisations, and one that is yours

Driehoek Health Group and **Rietvlei District Health Service** are fictional and carry the teaching in volume 1. Every figure attributed to either is illustrative.

Ravensmere Health Network is the populated repository that chapter 21 reads end to end. The chapter treats its defects as the teaching material, because each one is a caricature of a documented real pattern.

Real organisations, vendors and regulators are named normally throughout, with sources and dates. No text, table or figure is reproduced from ArchiMate, TOGAF, HL7, SNOMED CT, ICD, DICOM, IHE, openEHR, ISO, IEC or any other standard or licensed model.

What is not here yet

Still to come	What it will be
`02 Workbook`	The TOGAF ADM phase by phase as numbered steps, printable and written in
`03 Excel templates`	Six: the empty repository, a baseline-to-target comparison tool, workshop capture, transcription forms, principles and standards, a maturity assessment
`04 PowerPoint`	Three: an executive briefing, empty ArchiMate 4 view templates, a workshop pack
`05 Worked example`	The populated Ravensmere repository and its 27 ArchiMate 4 views

Until those arrive, the guide stands on its own. Chapter 22 is written so that an architect can act on it without the workbook, and chapter 20 gives the modelling patterns without the view templates.